Pegylated liposomal doxorubicin · every 4 weeks

National Cancer Control Programme (NCCP) 00205 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your infusion and access plan

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

STEP 1

Before the first visit

Confirm the indication, baseline bloods, required cardiac checks, medicines review and consent before the first infusion.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

The first infusion starts at 1 mg per minute. Later infusions may take 60 minutes if there was no reaction; a previous reaction requires a modified supervised schedule. Staff observe for reactions. Report burning, pain, swelling or leakage at the infusion site immediately.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

The first infusion starts at 1 mg per minute. Later infusions may take 60 minutes if there was no reaction; a previous reaction requires a modified supervised schedule. A port is optional access, not a requirement for a take-home pump. This regimen has no routine ambulatory pump.

Premedication

No routine premedicines are specified. Low-risk antiemetic support is separate. Infusion reactions may require a different supervised plan for future doses.

Sources: NCCP 00205 v8 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.